



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : منال صديق اندراوس ارمانوس

المحافظة : القاهرة الحي / المدينة : عين شمس العنوان : ...

رقم البطاقة : 28205150201906 رقم التليفون : 01224936911 تاريخ تسجيل الإستمارة : 03-10-2026

العمر (تاريخ الميلاد) : 15-05-1982 المهنة : ربه منزل

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات أو أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (.....) التوقيع: (.....)

المقرر بما فيه (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام مزيد من الأبحاث و الاشعات [7954] قسم العلاج : الاورام

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني الوظيفة: التوقيع:
الاسم: د/ندى حسن الوظيفة: التوقيع:
الاسم: د/نرمين رمضان الوظيفة: التوقيع:
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

..... د/..... د/..... د/..... د/.....

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى : قسم :
جهة العلاج : نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: منال صديق اندراوس ارمانويس National ID: 28205150201906	
---	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
---	---

وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حمید بالثدى و تحتاج الى Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



جمهورية مصر العربية
بطاقة تحقيق الشخصية

منال

صديق اندراوس ارمانوس

٥٠ محمد النجارى خلف محطة صفر-هاكوس

اول الرمل - الاسكندرية

١٩٨٢/٠٤/١٥

٢٨٢.٥١٥.٢.١٩.٦

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طب
٢٠١٥



٢٠٢٢/٠٦ ٢٨٢.٥١٥.٢.١٩.٦

حاصلة على دبلوم معهد فنى صناعى

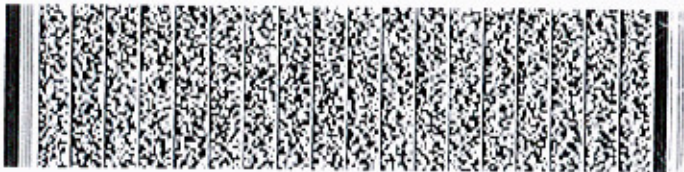
أنسة

مسيحية

أنثى



البطاقة سارية حتى ٢٠٢٩/٠٦/١٨





Ministry of Health and Population
Kingdom of Saudi Arabia

وزارة الصحة والسكان
المملكة العربية السعودية



A-Patient ID:

1- Name:

فاطمة محمد بن عبد الله

Sex:

MDT form
3- Medical number:

Female

83216125

Old Med:

2020-2024

2-ID number: 28205150201906

Patient Age: 45

Family history:

None

Phone:

01224930921

BMI:

Marital status:

Married

NRS Score:

0.5

GROUP:

Low

Co morbidity:

None

Menopausal status:

Postmenopausal

Initial STAGE:

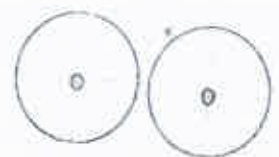
Stage I

REASON OF MDT:

Reason of not
A- Tumor

Date of MDT:

2025-04-03



Breast Imaging:

It breast UOQ 2 masses 30 x17 mm 9 mm from skin & 30 m from
rt nipple
solid well defined mass 22 x11 mm
no pathological axillary LN

Clinical data

Mets Workup:
No metastases

BX IDC	ER 8/8	PR 7/8	Her 2 Neu 0	SISH	KI67 10
1					
2					
		N 0		M 0	

Sonomammography

It breast UOQ 2 masses 30 x17 mm 9 mm from skin & 30 m from
rt nipple
solid well defined mass 22 x11 mm
no pathological axillary LN

PATIENT HISTORY

cesm: It breast BIRADS 6 bifocal UOQ malignant masses
rt breast BI RADS 2 intramammary LN
pt ended 4 taxotere+4 ac
LT WLE+SLNB: invasive mucinous carcinoma low grade YPT2(3CM)
free resection margin & extra margin
partial pathological response
LN 0/4

MDT OUTCOME & PLAN

- PORT
- ADJUVANT ZOLADEX AI
- ZOMETA/6 MONTHS

DICAL ONCOLOGY

SIGNATURE

RADIOLOGY

RADIOLOGY

Name: منال صديق اندراوس		
MR. : 3015		
Detailed Medical Report		
		السيد
تحية طيبة وبعد,,		
إيماء للطلب المقدم من	بشأن تقرير طبي للمريض	
بقسم:	رقم طبي:	
نتشرف بأن نوضح فيما يلي تقريراً عن حالته		
تاريخ وساعة الدخول :	تاريخ وساعة الخروج :	
وبعد الكشف والفحوصات والمتابعة وجد انه		
The patient is a 43-year-old individual with a diagnosis of left-sided invasive ductal carcinoma of the breast, estrogen-receptor score 8/8, progesterone-receptor score 7/8, HER2-negative, clinical stage T2N0M0. Neoadjuvant chemotherapy consisting of four cycles of doxorubicin/cyclophosphamide (AC) followed by four cycles of docetaxel (TXT) was administered, after which the patient underwent left wide local excision with sentinel lymph node biopsy revealing a pathological stage ypT3, 3 cm tumor, ypN0 (0/4 nodes). Post-operative imaging demonstrated a partial radiological response. Adjuvant endocrine therapy with letrozole (FEMARA) and a gonadotropin-releasing hormone agonist (ZOLADEX) was initiated in April 2025; the patient also receives calcium supplementation (1 tablet daily) and zoledronic acid (Zometa 4 mg) for bone health. Current plan includes continuation of adjuvant therapy and routine follow-up. The clinical pharmacy note records the patient's weight as 80 kg, serum creatinine 0.62 mg/dL, and calcium level 9.93 mg/dL while on the regimen of zoledronic acid (Zometa), goserelin acetate (Zoladex), and letrozole (Femara). No changes to the medication regimen were noted.		
تحريراً في ,		
التاريخ :	الوقت :	18-09-2026
الطبيب المعالج :	ايه مدحت بدوي محمد	
يعتمد ,		

Patient Name : الأستاذة / منال صديق الشراوس أرمانتوس
Patient No. : 28205150201906
Path No. : 1970/24
Sex : Female
Age : 42 Y
Referring Doc. :
Request No. : 13005091
Request Date : 10/06/2024
Verification Date : 01/07/2024
Financial Category : أورام دار السلام - مبادئ
Department : Out Patient
IHC No. :
Registered by : marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Amended report
Clinical Data: Left breast mass.

Nature of Specimen: Tru-cut biopsy, collected at 10/6/2024, 10:30 am.

Technique:

- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:

- * IPT (Positive control showed appropriate reactivity):
- E-cadherin: Positive.
- ER: Strong nuclear reaction in 80% of tumor cells (score 8/8).
- PR: Strong nuclear reaction in 60% of tumor cells (score 7/8).
- Her2: No membrane reaction in the tumor cells (score 0).
- Ki67: 10% nuclear reaction of tumor cells.

Conclusion:

Left breast mass, Tru-cut biopsy:
Invasive duct carcinoma, Grade II.

IPT:

- ER: Positive (score 8/8).
- PR: Positive (score 7/8).
- Her2: Negative (score 0).
- Ki67: Positive in 10% of tumor cells.

Verified by :

Dr. Sagda El-Kady

Printing Date : 02/07/2024 08:55:54

Page 1 of 2

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Patient Name الأستاذة / منال صديق اندراوس ارمانيوس
Patient No. 28205150201906
Path No. 1970/24
Sex Female
Age 42 Y
Referring Doc.

Request No. 13005091
Request Date 10/06/2024
Verification Date 01/07/2024
Financial Category اورام دار السلام - مياثره
Department Out Patient

Registered by .marwa mohamed yours

IMMUNOHISTOCHEMISTRY REPORT

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is intense and within < 10% of the invasive tumor cells.

Signature
Dr. Marwa Mahmoud.
Dr. Salma Tarek.

Verified by :
Dr. Sagda El-Kady

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Page 2 of 2

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غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt



Patient Name: Manal Seddik Andraws Armanios	Date : 03/06/2024
Exam : ultrasound BREAST (BOTH SIDE)	Age : 42 Year(s)
Medical ID: 28205150201906	Pat. ID: 54520

**DIGITAL BILATERAL SOFT TISSUE MAMMOGRAPHY AND
COMPLEMENTARY ULTRASOUND**

Indication:

Left breast palpable mass.

Technique

- Bilateral MLO and CC views
- Bilateral 3D MLO DBT (Digital Breast tomosynthesis)
- Bilateral breast ultrasound

Report:

- Left Breast:
 - Scattered fibroglandular density (ACR B).
 - Left breast UOQ region shows two partially obscured medium density masses lesions.
 - Complementary U/S left 3 o'clock zone B two adjacent masses lesions:
 - Solid hypoechoic spiculated mass lesion measuring 30 x 17 mm, 9 mm from skin and 30 mm from the nipple (closer to the nipple than the to be mentioned).
 - Solid well defined hypoechoic mass lesion measuring 22 x 11mm.
 - Normal skin thickness, and nipple-areola complex with average caliber of the retro-areolar central ducts.
 - No pathological axillary lymph nodes.
 - No pathological enlarged infra or supraclavicular lymph nodes.
- Right Breast:
 - Scattered fibroglandular density (ACR B)
 - No suspicious micro calcifications or architectural distortion detected.
 - Complementary ultrasound no cystic or solid mass lesions.
 - Normal skin thickness, and nipple-areola complex with average caliber of the retro-areolar central ducts.
 - Nonspecific axillary LN.

Opinion:

- Left breast suspicious mass lesion (BIRADS 4) for histopathological examination .



Patient Name: Manal Seddik Andraws Armanios	Date : 03/06/2024
Exam : ultrasound BREAST (BOTH SIDE)	Age : 42 Year(s)
Medical ID: 28205150201906	Pat. ID: 54520

- Adjacent left breast benign looking lesion BIRADS-3 for better histopathological assessment.
- Normal sonomammography of the right breast (BIRADS 1)

Signature Dr. Mohamed Mohsen

03/06/2024 4:48 PM



Patient Name: Maged Beddik Andraw's Amrany	Date: 02/07/2024
Exam: C.T ABDOMEN & PELVIS	Age: 42 Year(s)
Medical ID: 2070/2024	Pat ID: 54520

NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS:

- Reported case of left breast cancer.

I Chest:

- Left breast two parenchymal nodular soft tissue lesions largest measures about 2.2 x 1.5 cm in axial dimensions. Associated prominent left axillary lymph node (level II), measures about 7 mm.
- No sizable suspicious pulmonary nodules, areas of ground glass attenuation, consolidation or cavitary lesions.
- No CT evidence of pathological mediastinal lymph node enlargement.
- No pleural or pericardial collection, pleural thickening or masses.
- No cardiac gross abnormalities.
- *Tiny hypodense nodules of both lobes of thyroid gland. For US correlation.*

II Abdomen & pelvis:

- The liver is average in size showing no definite sizable focal lesions on non-contrast basis.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal CT appearance of the spleen, pancreas, kidneys and suprarenals glands, IVC and aorta.
- Averagely filled urinary bladder showing no sizable diverticular outpouching.
- No pathological abdominal or pelvic lymph node enlargement.
- No ascites.

III The Examined Skeleton:

- No sizable worrisome lytic or osseous deposits.

Opinion:



Patient Name: Manal Seddik Andrews Armanios	Date: 02/07/2024
Exam: C.T ABDOMEN & PELVIS	Age: 42 Years
Medical ID: 2070/2024	Pel ID: 54520

- Left breast two parenchymal nodular soft tissue lesions. For sonomammography correlation.
- No sizable pulmonary, hepatic or osseous deposits on non contrast basis.

Dr. Ali Elsisy, MSc.
Dr. Amr Farouk, M.D.



Patient Name: Manal Seddik Andraws Armanios	Date : 28/01/2025
Exam : MAMMOGRAPHY BOTH	Age : 42 Year(s)
Medical ID: 2070/2024	Pat. ID: 54520

**DIGITAL BILATERAL SOFT TISSUE MAMMOGRAPHY AND
COMPLEMENTARY ULTRASOUND**

Indication:

Left breast malignant masses after NACT.

Report:

- Heterogeneous fibroglandular densities (ACR c).
- **Left Breast:**
 - Left breast UOQ region shows two metallic marking clips with associated partially obscured medium density masses. Complementary U/S left 3 o'clock zone B two adjacent masses lesions:
 - Regression of size of spiculated mass measuring 13x11mm compared to 30x17mm, 9 mm from skin and 30 mm from the nipple (closer to the nipple).
 - Stationary well defined hypoechoic mass lesion measuring 21x10mm compared to 22 x 11mm. (Likely infiltrated IMLN)
 - Normal skin thickness, and nipple-areola complex with average caliber of the retro-areolar central ducts.
 - No pathological axillary lymph nodes.
 - No pathological enlarged infra or supraclavicular lymph nodes.
- **Right Breast:**
 - Scattered fibroglandular density (ACR B)
 - No suspicious micro calcifications or architectural distortion detected.
 - Complementary ultrasound no cystic or solid mass lesions.
 - Normal skin thickness, and nipple-areola complex with average caliber of the retro-areolar central ducts.
 - Nonspecific axillary LN.

Opinion:

- Post NACT status of Left breast malignant masses showing partial response (BIRADS 6).
- Normal sonomammography of the right breast (BIRADS 1)

Best regards,
Dr. Amira H Radwan MD,
Breast Imaging consultant,
NCL, Cairo University
28/01/2025 5:01 PM

Patient Name : الأستاذة / منال صديق الشراوس أرمانيوس
Patient No. : 28205150201906
Path No. : 701/25
Sex : Female
Age : 42 Y
Referring Doc :

Request No. : 13003149
Request Date : 17/02/2025
Verification Date : 09/03/2025
Financial Category : أورام دار السلام - ميثارد
Department : Out Patient
Registered by : Mrs. Mayada Hegazy

Pathology Report

Clinical Data: Left breast cancer, post NACT.
Nature Specimen: Left WLE and SLNs.

Gross Examination:

- 1- Left WLE specimen measured 7x4x2 cm, no covering skin ellipse.
 - * Tumor extent: A wired firm grayish white tumor bed site
 - Size: 3x2x2 cm.
 - * Distance from margins: encroaching the superior margin.
 - * There was another grayish white glistening nodule measured 1.5x1x1 cm.
- 2- Left SLNs: A fibro fatty tissue piece measured 4x4 cm entangling 4 nodules.
- 3- Extra superiomedial margin: Fibrofatty tissue piece measured 6x3x2 cm, entangling grayish white area on side against to the suture.
 - * Frozen section:
 - Free SLNS (0/4).
 - Superior margin was flushed grossly so extramargin is requested then extra superior medial margin is received and free extramargin and free other margins.

Microscopic Examination:

Left breast, WLE and SLNs biopsy:

- * Histological Type: Invasive mucinous carcinoma (low grade).
- * Ductal carcinoma in situ: No.
- * Lymph node involvement: Free SLNs (0/4).
- * Angioinvasion: No.
- * Perineural invasion: No.
- * Resection Margins: Free margins and free extramargin.
- * Histologic response to therapy: Partial pathological response.
- * Microcalcifications: No.

Verified by :

Printing Date : 09/03/2025 14:04:54

Page 1 of 2

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الاسم : الأستاذة / منال مصطفى اندراوس ارحمانيوس :
: 28205150201906
: 701/25
: Female
: 42 Y

Request No. : 13003149
Request Date : 17/02/2025
Verification Date : 09/03/2025
Financial Category : مريض - أورام دار السلام
Department : Out Patient
Registered by : Mrs.Mayada Hegazy

g Doc :

Pathology Report

ional pathologic findings: No.

osis:
reast, WLE and SLNs Biopsy:
sive mucinous carcinoma (low grade).
a resection Margins and free extramargin.
tial pathological response to therapy.
ls were negative (0/4).
ge: ypT2, ypN0.
B: 2.064, RCB class II.

Signature
Dr. Rana Fathy.
Dr. Menna Nabil.

Verified by :

Printing By : Mrs.Mayada Hegazy

Printing Date : 09/03/2025 14:04:54

Page 2 of 2

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt





Patient Name : الأستاذة / منال صديق الشراوى ارمانيوس
Patient No. : 28205150201906
Path No. : 1970/24
Sex : Female
Age : 42 Y
Refereing Doc :

Request No. : 13005091
Request Date : 10/06/2024
Verification Date : 25/06/2024
Financial Category : أورام دار السلام - مبادره
Department : Out Patient
Registered by : marwa mohamed youss

Pathology Report

(Amended Report)

Clinical data: Left breast mass

Nature of specimen

Tru cut biopsy, collected at 10/6/2024, 10:30 am

Gross examination

Multiple grayish white tissue cores up to 1.5 cm, totally submitted.

Microscopic examination

- * Histological type: Invasive lobular carcinoma.
- * Grade (differentiation +nuclear pleomorphism+ mitosis): (3+2+2) Grade II
- * DCIS: No
- * Angioinvasion: Couldn't be detected in the received biopsy
- * Perineural invasion: No

Diagnosis

Left breast mass, Tru cut biopsy:

Invasive Duct carcinoma, Grade II.

For further IHC (ER, PR, HER2, Ki67)

Comment: The report is amended after performing immunophenotyping for E-cadherin , and revealed positive results, compatible with duct carcinoma.

Signature

Dr. Salma Tarek
Dr.Heba Hassan

Dr. Marwa Mahmoud

Verified by :

Dr.Heba Hasan

منال صديق الشراوى
ا.م.د. منال صديق الشراوى
10/6/2024





Patient Name : الأستاذة / منال صديق النراوس ارمانيوس
Patient No. : 28205150201906
Path No. : 1970/24
Sex : Female
Age : 42 Y
Referring Doc. :

Request No. : 13005091
Request Date : 10/06/2024
Verification Date : 01/07/2024
Financial Category : أورام دار السلام - ميانره
Department : Out Patient
IHC No. :
Registered by : marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Amended report

Clinical Data: Left breast mass.

Nature of Specimen: Tru-cut biopsy, collected at 10/6/2024, 10:30 am.

Technique:

- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:

- * IPT (Positive control showed appropriate reactivity):
- E-cadherin: Positive.
- ER: Strong nuclear reaction in 80% of tumor cells (score 8/8).
- PR: Strong nuclear reaction in 60% of tumor cells (score 7/8).
- Her2: No membrane reaction in the tumor cells (score 0).
- Ki67: 10% nuclear reaction of tumor cells.

Conclusion:

Left breast mass, Tru-cut biopsy:

Invasive duct carcinoma, Grade II.

IPT:

- ER: Positive (score 8/8).
- PR: Positive (score 7/8).
- Her2: Negative (score 0).
- Ki67: Positive in 10% of tumor cells.

Verified by :

Dr. Sagda El-Kady

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Page 1 of 2

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